

Trauma and medical evaluation

Evaluator, consent, and limited purpose

Dr. Elian Sore is a licensed clinician asked to document A.R.M.'s reported symptoms and observable findings. The evaluation occurred on May 22 and May 29, 2024 with a qualified Kalyrian-language interpreter. A.R.M. consented to examination and release of this report to the immigration court after counsel explained that it would be available to DHS.

The report is not treatment advice and does not determine who caused an injury. It evaluates whether current findings are consistent, inconsistent, or nonspecific in relation to the history A.R.M. supplied. Sore reviewed the CAT application, A.R.M.'s declaration, the May 21, 2023 clinic notation, and photographs whose chain of custody is described elsewhere. No unnecessary personal identifier or expanded name appears.

History supplied for evaluation

A.R.M. reported being held for two nights in May 2023, struck on the ribs and thighs, restrained at the wrists, deprived of ordinary sleep, and questioned about copied procurement ledgers.

A.R.M. described bruising after release, ankle swelling, transient pain with deep breathing, nightmares, and heightened alertness near uniforms or mechanical door sounds.

The evaluator did not assume this account was true. Questions separated memories of the event from facts learned later through family members. A.R.M. consistently distinguished what was seen, felt, inferred, and reported by S.M. The interview disclosed no loss of consciousness, fracture diagnosis, surgery, or hospital admission. Those negative details limit the conclusions.

Records and source reliability

The May 21, 2023 clinic notation records registration at 14:20, limited left-shoulder elevation, right-ankle swelling, rib tenderness, hoarse voice, and advice to return if breathing worsened. It contains no diagnosis of the person or institution responsible. The copy was authenticated through a clinic-source statement and comparison with the family packet. Its translation identifies two ambiguous abbreviations and preserves both possible readings.

Photographs show discoloration and swelling but contain no embedded location data. A family log fixes the date by reference to a message exchange and clinic visit. Sore treats the images as corroborative of appearance, not as proof of cause or exact timing. No later treatment file was available beyond ordinary primary-care notes unrelated to the claimed detention.

Physical examination

The 2024 examination found a narrow, healed area of reduced pigmentation near the left wrist, mild tenderness at one rib margin without instability, and full ankle range of motion. There was no current swelling, neurological deficit, or limitation in ordinary gait. Vital signs and basic respiratory observations were within expected ranges for the examination setting.

These findings are compatible with many causes and are not diagnostic of restraint or assault. The passage of one year reduces the ability to distinguish mechanisms. The wrist finding could be consistent with prior friction or pressure; the rib tenderness is nonspecific. Sore records both consistency and uncertainty rather than converting an observation into an attribution.

Psychological symptom interview

A.R.M. described recurrent dreams involving the metal door sound, avoidance of procurement offices, increased startle response, difficulty concentrating after official correspondence, and guilt about exposing family members to attention. Symptoms reportedly began after the May 2023 event and fluctuated after departure from Kalyra. A.R.M. continued daily activities and attended all scheduled legal appointments.

Screening responses met a threshold for further trauma-focused assessment but did not establish a standalone diagnosis in this limited evaluation. Cultural and interpretation factors can affect word choice, and litigation stress may intensify symptoms. The report therefore describes the pattern and functional effect without claiming that symptom presentation proves the underlying account.

Consistency analysis

The reported timing of bruising, swelling, and rib pain is consistent with the clinic notation and family log. The wrist observation one year later is also compatible with prior pressure, though not specific to it. The absence of a fracture or hospital record does not contradict the described soft-tissue injuries, but neither does it independently confirm detention.

The symptom narrative remained materially stable across two interviews. Minor differences concerned whether a threat preceded or followed a change in questioning topic. That sequence does not alter the medical assessment and is preserved for the factfinder. Sore assigns no legal credibility label and expresses no opinion on CAT probability.

Alternative explanations and limits

Possible alternative causes include occupational strain, ordinary accidental injury, preexisting anxiety, migration stress, and sleep disruption associated with litigation. The available notes do not document a comparable wrist or rib complaint before May 2023, but absence from a limited file is not proof that none existed. No toxicology, imaging, or contemporaneous specialist examination was performed.

Sore did not interview Unit Four personnel, Harbor League members, or the original clinic worker. The evaluator cannot date the healed wrist finding to a particular day. Translation and memory limitations are disclosed. Conclusions are framed as consistency judgments, not causal certainty.

Function and treatment history

A.R.M. reported avoiding crowded transit platforms, waking several times per week, and needing extra time to review official documents. At the same time, A.R.M. maintained housing, assisted counsel with exhibit organization, and communicated regularly with S.M. The report records both impairment and retained function.

Brief supportive counseling occurred through a community clinic beginning in March 2024. The notes emphasize grounding techniques and sleep routine; they do not contain a forensic conclusion. No medication change is attributed to the claimed events. Future treatment decisions belong to a treating clinician and do not determine the adjudicative question.

Interpretation safeguards

Interpreter code MED-K14 affirmed fluency in Kalyrian and English medical vocabulary and disclosed no relationship to A.R.M. The interpreter translated questions in the first person and flagged two terms that could mean either watchfulness or fear. Both meanings were explored. The evaluator did not ask the interpreter to assess truthfulness.

A.R.M. identified the visit as May 21 before the source was displayed. The source then fixed the registration time at 14:20 and the observed sides as right ankle and left shoulder. Interview notes preserve the question, response, and source comparison. The interpreter declaration and source-review log are retained as components of this report.

Relationship to future risk

Past physical and psychological findings may provide context, but this evaluation cannot predict what will occur after a return to Kalyra. Sore has no expertise in registry practices, security units, contractor networks, or government protection. The report therefore offers no opinion on relocation, official acquiescence, or aggregate probability.

If the factfinder credits the account, the findings are medically compatible with portions of it. If the account is not credited, the findings do not independently identify a perpetrator. This conditional statement is the outer boundary of the evaluator's evidence.

Materials considered and omitted

Materials considered were the application chronology, A.R.M.'s declaration, the family packet, translated clinic notation, photographs, two interview sessions, and limited counseling notes. The country report was reviewed only to avoid using the same terminology for medical and institutional questions. No document, interview, or clinical observation arising after the second evaluation session on May 29, 2024 was part of this evaluation.

No raw genetic, biometric, passport, or address data were requested. No recording of the clinical sessions is included. The report identifies exactly which conclusions depend on self-report and which rest on observation or a contemporaneous notation.

Attestation

Sore attests that the examination and report were completed for this proceeding, that compensation was a fixed fee, and that the stated limitations are complete. The report may be tested through cross-examination. The evaluator will correct any transcription error but will not alter an opinion merely to favor the retaining party.

Signed June 7, 2024. The twelve-page packet includes the consent summary, interpreter declaration, record list, findings, consistency analysis, limitation statement, and attestation. Its evidentiary source cutoff is May 29, 2024. It contains no claim about the ultimate legal result and no identifying information beyond A.R.M.'s initials.